

DATE OF DISCHARGE:

DOCTOR:

DATE OF

UUID: 7AE1E138-C824-4547-AA87-F126748DA14D  
TCGA-FJ-A3Z9-01A-PR

Redacted

Pathology Consultation Report

Final

CASE:

**ANATOMIC PATHOLOGY DIAGNOSIS:**

Urinary bladder, left lateral wall, transurethral resection of bladder tumor

- Papillary urothelial carcinoma, high grade, invasive with squamous and glandular differentiation
- Tumor invades extensively into lamina propria and muscularis mucosa with focal invasion into the muscularis propria, approximately 5% of tumor with micropapillary pattern
- Lymphovascular invasion noted

Prostatic urethra, transurethral biopsy

- Benign urothelium with chronic inflammation
- Chronic prostatitis with calcifications
- Stromal nodule
- No atypia or malignancy

ICD-O-3

carcinoma, papillary urothelial

8/30/13

Site: bladder, wall, lateral C67.2

fw  
9/28/12

**PATHOLOGIST:**

I have reviewed this material and confirm the report. Released by electronic signature on:

**MATERIAL:**

Bladder tumor, left lateral wall  
Prostatic urethra

Page 1 of 3

**SURGICAL PATHOLOGY**

w/ squamous + glandular differentiation

| Criteria                        | Yes     | No |
|---------------------------------|---------|----|
| Diagnostic Discrepancy          |         |    |
| Primary Tumor Site Discrepancy  | X       |    |
| HPAA Discrepancy                |         |    |
| Prior Malignancy History        |         | X  |
| Local/Synchronous Primary Noted |         | X  |
| Case Is (Circle):               |         |    |
| Reviewer Initials               | fw      |    |
| Date Reviewed                   | 7/13/12 |    |

fw  
8/27/13

DATE OF DISCHARGE:

DOCTOR:

DATE OF SERVICE:

Pathology Consultation Report

Final

CASE:

HISTORY:

male with pre-op diagnosis is bladder cancer.

GROSS:

A - Received in formalin labeled "bladder tumor, left lateral wall" is a 5.0 x 4.5 x 2.3 cm aggregate of multiple pieces of irregular tan-pink friable tissue which is submitted in toto in cassettes A1-A14.

B - Received in formalin labeled "prostatic urethra" are two irregular tan-pink rubbery tissues both averaging 2.0 x 0.6 x 0.4 cm. Each specimen is bisected and submitted in its entirety in cassettes B1-B2.

MICROSCOPIC:

Microscopic examination of the transurethral resection of the left lateral bladder wall tumor reveals a high grade papillary urothelial carcinoma which in some areas has an inverted pattern. The tumor shows extensive multifocal invasion into the lamina propria and the muscularis propria with focal invasion into the muscularis propria. Additionally, lymphovascular invasion is also seen. Approximately 5% of the tumor also has a micropapillary pattern. The tumor additionally shows multifocal extensive areas of squamous as well as glandular differentiation.

Examination of the prostatic urethral tissue reveals benign urothelium with underlying chronic inflammation and benign prostatic tissue with some mild chronic prostatitis as well as a stromal nodule. Urothelial dysplasia or malignancy is not noted. Prostatic intraepithelial neoplasia or malignancy is seen.

Page 2 of 3

SURGICAL PATHOLOGY

  
DATE OF DISCHARGE:

  
DOCTOR:

DATE OF SERVICE:

Pathology Consultation Report

Final

CASE:

COMMENT:

The vascular invasion is confirmed on D240 and CD31 staining. This is distinct from the micropapillary pattern which is noted in approximately 5% of the invasive tumor.

INTRADEPARTMENTAL CONSULTATION:

  
Page 3 of 3

SURGICAL PATHOLOGY